



CHIROPRACTIC
SLEEP ACADEMY

CHIROPRACTIC SLEEP ACADEMY® · POSITION PAPER

Primary Respiratory Biomechanics & the Skull-Down Model

The Whole-Body Structural Dimension of Sleep & Airway Care

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EXECUTIVE SUMMARY

An Under-Addressed Dimension

Snoring, insomnia, fatigue, airway collapse, and obstructive sleep apnea are usually approached from the top down (CPAP) or the jaw forward (oral appliances).

Both are legitimate, effective tools. But there is an **under-addressed dimension**: sleep and airway function are shaped by the **whole-body structure the airway sits in — from the skull to the feet**. This paper sets out the **Skull-Down model** and the Doctor of Chiropractic's role in that structural dimension — measured, and coordinated with medicine and dentistry as peers.

SECTION 1

The Biomechanical Basis of Sleep & Airway

Airway patency is influenced by many structures, not one:

- cranial mechanics and rhythm
- tongue posture
- maxillary dynamics
- cervical curve integrity
- thoracic expansion
- diaphragmatic descent
- pelvic mechanics
- sacral-dural relationships
- and even foot posture and ground-reaction forces

The whole chain matters: foot posture and gait **may influence** airway mechanics — through proprioceptive input, spinal rhythm, pelvic balance, diaphragmatic recruitment, and rib-cage excursion. The point is not that the feet "determine" the airway; it is that a **whole-body system deserves a whole-body assessment**.

These relationships are the Skull-Down model — a clinical framework we assess and measure, not settled physiology.

SECTION 2

The Skull-Down Model

The Skull-Down model assesses the airway within the whole structural chain:

- **Primary Respiratory Biomechanics (PRB)** — the coupled cranial, spinal, and diaphragmatic movement of breathing
- pelvis, gait, and the foot-ankle complex
- cervical lordosis
- cranial mechanics and the **Reciprocal Tension Membrane (RTM)** — the dural-membrane system (falx/tentorium), a cranial concept CSA works within
- viscerocranial (aperture) considerations

This is presented as an integrative **clinical model** — a lens to assess and measure, not a set of proven mechanisms. It is a whole-body structural framework, **not** a claim to replace airway medicine.

SECTION 3

What CPAP & Oral Appliances Do — and Don't — Address

CPAP and oral appliances **work** — they hold the airway open, and for many patients they are the right tool. Their well-documented limit is **use**: a large share of patients can't or won't tolerate CPAP, and appliances don't fit every case. And neither addresses the **structure around the airway** — the cranial, spinal, and pelvic mechanics that shape how a body breathes. That structural gap is where the Doctor of Chiropractic contributes — **alongside** these tools, not instead of them.

SECTION 4

Why the Doctor of Chiropractic Is Equipped for the Structural Dimension

The Doctor of Chiropractic brings training that fits this dimension:

- whole-body structural assessment
- cranial and spinal mechanics
- neuromuscular and postural evaluation
- hands-on capability
- a conservative, whole-person model

In California, the Doctor of Chiropractic **diagnoses and manages** sleep apnea, snoring, and insomnia within full scope. The DC is **uniquely equipped to lead the structural dimension** of the case — and coordinates the dentist, physician, and ENT **as peers**. This is leadership of the whole-body structural work, not a claim to be the only discipline that matters.

SECTION 5

The Mandate — Measured, Honest, Collaborative

The Skull-Down model asks one thing of the profession: assess the **whole** body a person breathes with, **measure** what changes (pre/post testing, including non-responders), and collaborate across licensure lines. Structural care is offered as an **early, whole-body dimension** of sleep and airway care — always measured, never a cure claim, never a replacement for CPAP, appliances, or surgery.

A body breathes. Our work is to make sure the structure lets it.

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